



WHO Draft Global Action Plan on Lead Mitigation

Draft for consultation

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INTRODUCTION

Lead poisoning is a global, preventable health crisis, with as many as 1 in 3 children globally (800 million) having blood lead levels $\geq 5 \mu\text{g/dL}$ (IHME 2019¹), the level at which immediate actions should be taken to identify and remove the source of exposure, as per World Health Organization recommendations (WHO 2021²). In 2023, lead exposure contributed to more than 3.5 million deaths and 71.5 million years lost to disability (disability-adjusted life years, or DALYs) and was the second leading environmental risk, after air pollution, and the eighth leading risk for global mortality according to the Institute for Health Metrics and Evaluation (IHME 2026³).

The Seventy-eighth World Health Assembly adopted resolution WHA78.27 (WHO 2025⁴) calling for a draft WHO Global Action Plan for lead mitigation. The Global Action Plan strengthens WHO's ongoing work and mandate, in line with the Fourteenth General Programme of Work, which emphasizes addressing health determinants and the root causes of ill health, including environmental degradation and air, water, and soil pollution and contamination.

CONSULTATIVE PROCESS

The draft Global Action Plan was developed through a consultative process involving Member States, civil society organizations, partners, technical experts and WHO staff at all three levels. Consultation also included WHO global networks such as the Global Chemicals and Health Network, Chemical Risk Assessment Network, poison centres, WHO Collaborating Centres in Children's Environmental Health, and regional expert consultations.

GUIDING FRAMEWORKS AND PRINCIPLES

The Global Action Plan aligns with the framework of the WHO Chemicals Road Map mandated by World Health Assembly resolution 69.4 (2016) to strengthen health sector engagement in the sound management of chemicals. Pursuant to World Health Assembly resolution 76.17 (2023), the WHO Secretariat, in consultation with the WHO Global Chemicals and Health Network of ministry of health focal points for chemicals, initiated an update of the WHO Chemicals Road Map to enhance health sector engagement in the new Global Framework on Chemicals- for a Planet Free of Harm from Chemicals and Waste (hereafter the Global Framework on Chemicals), adopted in September 2023 at the Fifth International Conference on Chemicals Management in Bonn, Germany. The Global Framework on Chemicals recognizes the WHO Chemicals Road Map and the health sector's critical role in the sound management of chemicals and waste.

¹ <https://www.unicef.org/reports/toxic-truth-childrens-exposure-to-lead-pollution-2020>

² <https://www.who.int/publications/i/item/9789240037045>

³ <https://jamanetwork.com/journals/jama/fullarticle/2846862>

⁴ https://apps.who.int/gb/ebwha/pdf_files/WHA78/A78_R27-en.pdf

Several Global Framework on Chemicals targets align closely with WHO's mandate to prevent lead poisoning. These include reducing the global burden of disease attributable to lead exposure; ensuring universal access to poison centres with essential capabilities (Target A6); strengthening national biomonitoring and surveillance of blood lead levels, particularly among high-risk populations (Target B7); and enhancing policy coherence across chemicals, waste, health, labour, and human rights agendas to support integrated prevention efforts (Target E6).

The WHO Chemicals Road Map identifies actions where the health sector has a lead or supporting role in chemical management.

STRUCTURE OF THE GLOBAL ACTION PLAN

The Plan is aligned with the four action areas of the WHO Chemicals Road Map to promote health sector engagement in: (1) Risk reduction, (2) Knowledge and evidence, (3) Institutional capacity, (4) Leadership and coordination. As in the WHO Chemicals Road Map, the four action areas are interlinked; to avoid duplication, each action is included only once.

Each action area includes objectives and proposed actions for:

- Member States
- WHO Secretariat
- Stakeholders

To support implementation of the Global Action Plan, WHO has also developed a Technical Package focused on identifying the most effective, evidence-based actions to take at country level to make progress on preventing lead poisoning.

ACTION AREA 1: RISK REDUCTION

Global Target: Reduce population exposure to lead.

Objective A: Strengthen and enforce policies to eliminate or control key sources.

Proposed actions for Member States:

- Review existing legal instruments, standards and data on lead exposure and prioritize lead sources that are both high risk and amenable to intervention within the country-specific context.
- Develop and implement policies, regulations or standards to eliminate lead-based paint and non-essential uses of lead.
- Promote safer design and manufacturing practices in consumer products, including through reformulation (e.g., paint, ceramics, cookware, cosmetics).
- Ensure standards and market surveillance for lead in consumer products, toys, ceramics, spices, cosmetics, cookware and food, coordinated through standards and customs agencies.

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- Regulate used lead-acid battery (ULAB) recycling and other lead-containing waste and enforce measures to ensure alignment with environmentally sound management.
- Strengthen occupational exposure controls, industrial hygiene and worker protection in high-risk sectors.
- Set or review emission standards.
- Reduce lead exposure via food through setting standards, strengthening surveillance and promoting safe processing, lead-free food contact materials and addressing contamination across the food chain.
- Address lead in drinking-water through prevention, mitigation and/or remediation measures in line with WHO guidelines and recommendations.
- Implement lead-safe housing programmes in high-risk areas.
- Undertake regulatory or non-regulatory actions to reduce releases of lead from industrial sources.
- Address historical deposits of lead in environmental media from past anthropogenic activities to prevent exposure to lead.

Proposed actions for WHO Secretariat:

- Support the implementation of the Global Action Plan through the development of normative, evidence-based technical implementation guidance such as the WHO technical package on lead poisoning prevention
- Assist countries to prioritize lead exposure sources based on disease burden, feasibility and policy-readiness
- Provide technical guidance on elimination of major exposure sources, such as through reformulation (e.g., paint, ceramics, cookware, cosmetics), and using local source data to inform lead exposure and risk assessments in routine pediatric care visits.
- Support Member States with regulatory development, stakeholder engagement and risk communication.
- Support and strengthen existing networks of poison centres
- Promote public awareness, education, communication and training on lead-containing products (paint, ULAB, ceramics, spices, cosmetics) and best practices involving lead including safe food and dietary practices to reduce exposure and absorption.
- Promote alignment with UN partners and other stakeholders to strengthen intersectoral collaboration and integration of the Global Action Plan.

Proposed actions for stakeholders:

- Promote WHO compendium interventions on lead exposure.
- Adopt and implement safer recycling, smelting, and manufacturing practices, including appropriate source, process, and release controls.

- Conduct regular occupational blood lead level (BLL) monitoring and ensure worker protections, including provision of protective equipment, engineering controls, and safe work procedures in facilities handling lead.
- Identify and promote lead-free or reduced lead alternative product reformulations.
- Support waste-management and comply with requirements for environmentally sound handling, transport, and storage of lead-containing materials (including ULABs, e-waste, scrap materials).
- Implement product take-back or extended producer responsibility (EPR) mechanisms where applicable (e.g., ULABs).
- Cease unsafe practices, such as open burning, crushing, or unregulated dismantling of lead-containing waste.
- Provide technical support and capacity building, especially for low-resource settings.
- Collaborate on awareness raising and behavioural change.

ACTION AREA 2: KNOWLEDGE AND EVIDENCE

Global Target: Strengthen data generation, research and monitoring.

Objective B: Improve identification of lead exposure sources and population health impacts

Proposed actions for Member States:

- Identify lead as a priority chemical for national action
- Conduct assessments to identify major exposure sources for vulnerable subpopulations, in particular, children, pregnant women and workers.
- Build national capacity in biomonitoring and laboratory systems for blood lead testing and surveillance to identify populations at risk, prioritize responses and track progress.
- Produce national burden of disease and economic impact analyses to inform prioritization.
- Collaborate internationally to track progress towards measurable health-based indicators and to share data and evidence towards demonstrating reduced lead exposure globally, including engaging in WHO networks (Global Chemicals and Health Network, Chemical Risk Assessment Network, Poisons Centres Networks).

Proposed actions for WHO Secretariat:

- Support capacity strengthening to generate timely, disaggregated data on blood lead levels, exposure sources and health impacts, and to track progress towards relevant SDGs.

- Provide strategic guidance and technical leadership on tools, and modelling platforms for source attribution, biomonitoring, burden estimation and economic assessment.
- Support development of region- and country-specific evidence and case studies on effective mitigation.
- Encourage scientific and interdisciplinary research that informs innovative solutions to prevent health impacts from lead exposure and related pollution.
- Facilitate sharing of methodologies and best practices through WHO networks.

Proposed actions for stakeholders:

- Conduct operational research and support national studies, particularly to fill data gaps.
- Strengthen academic and research partnerships, emphasizing LMIC research leadership.
- Support development of public datasets, disaggregated by age and sex, on lead exposure and impacts.

ACTION AREA 3: INSTITUTIONAL CAPACITY

Global Target: Improve surveillance, clinical management and health-system readiness.

Objective C: Strengthen national surveillance and health-sector response

Proposed actions for Member States:

- Establish or expand routine blood lead level surveillance targeting priority populations.
- Adopt, adapt and strengthen implementation of WHO Guidelines for the clinical management of lead exposure.
- Integrate exposure screening, risk assessment, counseling and follow-up into primary health care, antenatal, paediatric and school health services.
- Build diagnostic and laboratory capability, including external quality assurance.
- Establish or strengthen poison centres, and enhance their role in detection, triage, clinical advice and public health alerts.
- Strengthen healthcare worker capacity to recognize, refer and manage lead exposure through continuing professional development and existing WHO–UNICEF training resources.
- Ensure availability of chelators on WHO List of Essential Medicines through essential medicines lists, procurement plans, supply chains and collaboration with networks and partners.

- Establish capacity for Pb incident/outbreak and emergency response, including rapid procurement of chelators.
- Establish referral pathways and case management standard operating procedures from community/primary healthcare to tertiary care.

Proposed actions for WHO Secretariat:

- Identify resource needs for technical guidance, tools and training for surveillance, laboratory quality assurance and clinical management.
- Develop, maintain and disseminate training materials, guidelines and online courses on lead poisoning prevention.
- Support primary health care integration for exposure screening and risk communication.

Proposed actions for stakeholders:

- Support healthcare worker training and continuing medical education.
- Strengthen laboratory networks and international reference systems.
- Support tele-toxicology, poison centre networks, professional associations and academic partners.

ACTION AREA 4: LEADERSHIP AND COORDINATION

Global Target: Strengthen multisectoral governance and cooperation.

Objective D: Enhance multisectoral coordination and national programme leadership

Proposed actions for Member States:

- Establish or strengthen national lead poisoning prevention programmes with multisectoral governance, national roadmaps, and monitoring frameworks.
- Include BLL testing, confirmatory diagnostics and chelation therapy in universal healthcare benefits and reimbursement schemes.
- Create interministerial coordination platforms across relevant sectors.
- Assess policies across government against the WHO Technical Package on lead poisoning prevention and address gaps identified.
- Promote public awareness and engagement, including participation in International Lead Poisoning Prevention Week.
- Strengthen regulatory enforcement and market surveillance (customs, product recalls).
- Implement Basel (e.g. ULABs) and Rotterdam (e.g. Pb fuel additives) Conventions.
- Engage in international initiatives such as the Global Alliance to Eliminate Lead Paint and the Partnership for a Lead-Free Future.

Proposed actions for WHO Secretariat:

- Leverage WHO's convening role to promote collaboration and strengthen multisectoral coordination, including health sector engagement in international initiatives, such as the Global Framework on Chemicals and the Intergovernmental Science Policy Panel on Chemicals, Waste and Pollution.
- Provide technical and strategic support for regulatory consistency and enforcement.
- Coordinate communication and awareness campaigns, including the annual International Lead Poisoning Prevention Week of action.
- Promote integration of chemical safety into universal health coverage, primary health care and emergency response frameworks.

Proposed actions for stakeholders:

- Support policy uptake and enforcement through industry cooperation, civil society advocacy, and donor engagement.
- Support community-level awareness and involvement in mitigation activities.
- Facilitate resource mobilization and partnerships.

COORDINATION, MONITORING AND EVALUATION

WHO will coordinate implementation and support Member States through existing reporting and accountability mechanisms. Countries may tailor actions to national context and capacity, with WHO and partners providing technical support for implementation and evaluation. Annual reporting may be integrated into existing environmental health or chemical safety reporting channels, with mid-term (2028/2029) and end-term (2030) reviews to assess progress and recalibrate actions.

Reporting to the World Health Assembly

As mandated by WHA Resolution 78.27, the draft Global Action Plan will be submitted to the Eightieth World Health Assembly in 2027 through the Executive Board. Progress reports will be submitted to the Eighty-second World Health Assembly (2029) and the Eighty-fourth World Health Assembly (2031).